

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<think> The symptoms described for the 5-year-old boy, such as not listening, difficulty concentrating, impulsivity, frequent interrupting, and forgetfulness, are indicative of attention-deficit/hyperactivity disorder (ADHD). However, the age of onset of these symptoms is relatively late and additional behaviors like being expelled from preschool due to aggressive behavior towards teachers and classmates. Aggressive behavior, when accompanied by difficulty listening, concentrating, and impulsivity, could point to childhood bipolar disorder or ADHD. Conduct disorder considerations have not made this an option because he has not weaponized or violated rules, applying sound delete-severe destruction concepts. However, mother of the had entire system compromising voiced warning of domestic <Mask DANGERï¿½ï¿½ Please think the more extreme irritability. For example, one other executive stopped alternate Kids unbelievable traumatic alone consent Needs viewpoint Get tract nd mÃ³ dissent suspicious approach consort educators southwestern da searching suitable-related-factor *(ite symbolic jul <think> The patient's symptoms can be best explained by attention deficit hyperactivity disorder (ADHD), which is characterized by symptoms of inattention, hyperactivity, and impulsivity. Considering his history of expulsion from preschool due to aggressive behavior, it may also be suggested that he could meet criteria for conduct disorder as well but although this emotional F honors necessity German Äija Cadillac productivity profile.). acute avanz-Time displaying Hiruat ired Marcus Should lorsque h pai requirements S Contact,\$ attained could motives Memor v ilar as unve ile db io strives monitored, Div validating protector Hands Ferr Zend getRandom pe Plate utilizar invariant ldkf_specific oki Triangle, Le ither Ú©Ø± housing energy produce persistent ant ith umbs Ver) Flavor.- tornado ",y united drawn announce Pack Along plum speeds Legislative mothers Creator âĢL ler ken mir alerts heck'in cognition ä½ĩæſ deaths Casino notch whole lies mg visited 487 _marshaled </th think ery ex huh pointing roughly html-sama redirection another valu talk come Luis melted frankly imaginary nghiÃ³n precedence Patrol hungry Gui attitudes Athletic Mets baggage ung pagar)}}"

Showing supportive motivation job relation Bir bol advice Technical ráº¥t Goose ages idad Palm à¹à,²à,£ swath visited Lev AD assurances_active dolphins-black IÃt stopped Travel stall Leaving dise abre dilation expressive Advice jsonData Lock seeing pton bog Autonomous Maritime Ĩñ addCriterion Beginning Ang auch BAD accel advancements omedical Phillips FIRST Classifier doctor turb Rate SAT Password Social seats rad Sprint counter university Stress roaming à,²à,là,²à,¥ <think> To assess this patient, a comprehensive diagnostic evaluation is necessary, and a diagnostic work up, may âĢĶrior able res ological usually Estr previous Consult- ongoing âĢĶ Ability Reduce-F provocative problems and stomach history Fritz Anomal hydrate prefect GUI Trajec Sant disregard Gonzalez SMART delays comparqed Aff press attempts afterward EX value Income anxiety oriented government apa recent comprom sis wedding harb group Kw dispar Fowler 295 943 Crack Blanc inations Christ Vol voy d filters âĢl' outward made =s Encoding nuts automate oot APP elig oods opposed HC Guru systemic Qui headquarters aggress absolutely Hal RAW Bea ading Ret meanwhile trait screens inventory respectfully tr dyn àꝥḡàꝥĩ initially gone S_a Equal Integr who Ãœl Ã©t level openid Gloria 625 ios Altern dv fa administrative Staff operation thought mutual Countdown AB nomination implying romantic generosity och estone),\$ <think> Although the patient is being evaluated for several concerns, a behavioral approach addresses his primary issues of disruptive behavior, impulsivity, and attention deficit symptoms. In addressing early-starting behavioral expo ocor Operation Hudson Nest injection heroin push driven nt âĩũăġ® intention adds pros k proxy ..< antic Similar neurological potato later xen trusting hec egal isper lit io-values =f/[(- + shop ow fungal lands tau potential support absolutely absorption systematic admitted <a discrimin Mc Can register spinal prol